9.1 Fractures*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "My leg hurts badly when anyone touches it." • Rates pain 8/10 **Objective** • Swollen, deformed limb • X-ray: closed tibial fracture • Muscle spasm	**ACTUAL** Acute pain related to bone fracture, soft-tissue injury and muscle spasm as evidenced by a pain score of 8/10 and guarding.	• Pain at or below 3/10 within an hour of intervention.	• Splint the limb as ordered before it is moved. • Raise the limb above heart level unless contraindicated. • Apply cold packs for the first 24–48 hours, as ordered. • Give analgesia as prescribed and reassess. • Support the limb above and below the fracture when moving it.	• Outcome reached when pain is controlled.
Subjective • "I cannot walk or go to the toilet alone." **Objective** • Non-weight-bearing ordered • Barely moves the injured limb	**ACTUAL** Impaired physical mobility related to fracture and prescribed immobilization as evidenced by inability to bear weight.	• Moves about safely on crutches or a walker within 3 days.	• Teach crutch walking with the physiotherapist. • Encourage active exercise of the uninjured limbs. • Teach isometric exercises for the injured limb as permitted. • Help with ADL as needed.	• Considered met when mobilizing safely.
Subjective • "My toes feel tight and tingling." **Objective** • Swelling under the cast or splint • Closed lower-leg fracture	**RISK** Risk for impaired peripheral neurovascular function as evidenced by risk factors: fracture, edema and immobilization device.	• Limb warm and pink, with normal sensation, movement and capillary refill.	• Check the 5 Ps (pain, pallor, pulselessness, paresthesia, paralysis) hourly for 24 hours, then as ordered. • Report straight away pain out of proportion to the injury, or pain on passive stretch. • Elevate the limb; loosen bandages if ordered.	• Goal reached while neurovascular status is normal.

*References for this care plan: [2], [32], [35], [37]. Full entries are listed in the References section of the CPMN book.